

Describe the usual frequency pattern for urination.

A. Frequency of urination depends on

1. Amount of fluid ingested
2. Personal habits
3. Availability of toilet facilities
4. Physical activities
5. Illness/infection

B. Frequency ranges from every 2-3 hours to every 8-12 hours

1. Each patient/resident has different urination needs
2. It is important to keep the patient's/resident's routine as normal as possible

List the observations to be made about urine.

A. Observations made about urine:

1. Color
2. Clarity
3. Odor
4. Amount

B. Information that should be reported to the licensed nurse

1. Urine that appears abnormal
 - a. Cloudy, stones, gravel, sediment
 - b. Pinkish or reddish tint
 - c. Dark color/concentration
2. Patient/resident complaints
 - a. Urgency
 - b. Burning on urination
 - c. Difficulty in urinating
 - d. Feeling of pressure in area of bladder
 - e. Frequency
 - f. Strong odor

Describe urinary incontinence and the need for immediate care.

A. Urinary incontinence

1. Inability to control the passage of urine from the bladder
 - a. Constant leakage
 - b. Occasional leakage when laughs, coughs or sneezes
 - c. No control
2. Causes
 - a. Central nervous system problems
 - b. Spinal cord injury
 - c. Aging
 - d. Confusion/disorientation
 - e. Medications
 - f. Weak pelvic muscles
 - g. Urinary tract infection
 - h. Prostate problems (male)
 - i. Prolapse of uterus and bladder (female)
 - j. Restraints
 - k. Immobility
 - l. Unanswered call lights
 - m. Not having signal light within reach
 - n. Urinary frequency/urgency
 - o. Failure to toilet frequently
3. Signs of possible need to go to the bathroom
 - a. Restlessness
 - b. Fidgeting
 - c. Pulling at clothes/undressing
 - d. Holding or pointing at genitals
 - e. Crying
4. Nursing measures
 - a. Record incontinent episodes
 - b. Answer call lights promptly
 - c. Promote normal elimination

Describe urinary incontinence and the need for immediate care.

B. Immediate care is important to prevent

1. Embarrassment, shame, anger, frustration, and depression
2. Development of odors
3. Patient/resident is uncomfortable
4. Skin breakdown
 - a. Infection
 - b. Irritation, redness or rashes
5. Disposable briefs are to be avoided as it may cause low self- esteem and skin irritation, and may encourage incontinence

C. Nursing measures

1. Record voiding
2. Promote normal elimination
3. Follow elimination training programs
4. Encourage easy to remove clothing
5. Provide good skin care & perineal care
6. Dry garments and linens
7. Observe for skin breakdown
8. Use incontinent products as directed
9. Maintain clean, pleasant environment

Describe the purpose and general rules of care for urinary catheters.

A. Definition of urinary catheter

1. Plastic or rubber tube used to drain or inject fluid through a body opening
2. Most commonly used to drain the bladder
3. One end is placed in the bladder and the other end is attached by tubing to a drainage bag
4. Catheters are inserted by licensed personnel using sterile technique

B. Types of indwelling catheters

- a. "Foley"
- b. Retention
- c. Suprapubic
- d. Straight catheter

C. Purpose of urinary catheter

1. Complete loss of bladder control
2. Urinary retention (inability to urinate)
3. Before, during, and after surgical procedures
4. Too weak or disabled to use commode, bedpan or bathroom
5. Prevention of urine contamination on wounds and pressure ulcers
6. Not for convenience

Describe the purpose and general rules of care for urinary catheters.

D. Complication resulting from catheters

1. Bladder infection
2. Blockage
3. General rules of catheter care
 - a. Tubing should not be kinked or compressed (urine should flow without obstruction). Coil tubing on the bed
 - b. Do not pull on catheter tubing
 - c. Keep drainage bag below level of the bladder
 - d. Drainage bag should be attached to the bed frame, not the side rail
 - e. Never allow bag to touch the floor
 - f. Catheter should be secured to the inner thigh
 - g. Clean the perineal area and around the catheter, soapy water is adequate (use the procedure for catheter care preferred by facility)
 - h. Drainage bag should be emptied and measured each shift (or more often if ordered/required)
 - i. Report patient/resident complaints promptly to the licensed nurse
 - j. Follow the rules of asepsis at all times, keep drainage systems closed
 - k. Check for leaks and report status
 - l. Use separate measuring containers for each patient/resident
 - m. Encourage fluids

E. Report observations

2. Color, clarity, odor, presence of particles

Describe the goals and methods for bowel and bladder training.

A. General Guidelines

1. Goal is to regain voluntary control of bowels and bladder
2. Follow standard precautions
3. Follow bowel and bladder training schedule
4. Offer toileting per facility protocol
5. Explain schedule to patient/resident
6. Encourage proper diet
7. Answer call light immediately
8. Provide for privacy
9. Assist with perineal care to prevent skin breakdown
10. Keep accurate record of elimination pattern following facility protocol

B. Bladder training

1. If having difficulty voiding
 - a. Have patient/resident lean forward slightly
 - b. Run water
 - c. Never rush patient/resident
2. Follow facility policy for clamping of urinary catheter

C. Bowel training

1. Suppository at a regular time
2. Increase fluids
3. Diet will affect regularity

Describe the normal pattern of bowel movements and reportable observations.

A. Each person is different and has own pattern

1. Frequency – daily to every 2 to 3 days
2. Time of day – morning or evening
3. The Nurse Assistant should note and report stool shape, size, frequency, color, consistency, complaints of pain with defecation, etc.

B. Notable observations

1. Stool shape
2. Size
3. Frequency
4. Color
5. Consistency
6. Amount
7. Complaints of pain with defecation

Describe the normal pattern of bowel movements and reportable observations.

C. Factors influencing bowel movements

1. Privacy – fear of others, hearing and/or smelling
2. Age – as patient/resident ages, more problems
3. Diet – need balanced diet with regular meals, food stimulates bowel movement
4. Fluids – need adequate fluid
5. Activity – stimulates
6. Medications – most tend to constipate
7. Personal habits
8. Disabilities

D. General rules for maintaining normal elimination patterns

1. Promptly provide the bedpan, urinal, or commode, or help the patient/resident to the bathroom when the request is made
2. Assist the patient/resident to get into normal position (as much as is possible or safe)
3. Cover the patient/resident for privacy and warmth
4. Remain nearby if the person is weak or frail
5. Place the signal light and toilet tissue nearby
6. Allow the person time to carry out the elimination process
7. If patient/resident has difficulty, ask them what kinds of things they did at home to assist them, i.e., running water, reading or looking at a magazine
8. Provide perineal care if needed
9. Offer the opportunity to eliminate at regular intervals

Explain the purpose of an ostomy and the care of patients/residents with an ostomy.

A. Definition of ostomy

1. Surgical creation of an artificial opening most commonly into colon or small intestine
2. Some ostomies are for urinary drainage, i.e. nephrostomy, ureterostomy
3. Urine or feces and flatus pass through opening
4. Allow for healing of intestine after surgery or disease
5. Can be temporary or permanent

B. Terms used with ostomy patients/residents

1. Stoma – a portion of the intestine brought to the surface of the abdomen to allow for drainage
 - a. Pink and moist/red like mucous membrane
 - b. Size and shape are different depending on area of intestine and patient/resident
 - c. Can bleed when cleaned
2. Peristomal skin – the skin around the stoma; it should be clean, intact and dry
3. Appliance – the wafer and pouch or bag that protects the patient's/resident's skin and collects the drainage

/// Explain the purpose of an ostomy and the care of patients/residents with an ostomy.

C. Ostomy types

1. Location depends on the disease or injury

2. Names according to location

a. Colostomy

1) Ascending and transverse

a) Stool is consistency of pureed liquid with slightly acidic content

b) Must wear pouch all the time

c) Chew food well with a lot of fluids

2) Descending and sigmoid

a) Stool formed and looks “normal”

b) Pattern of stool drainage - every day or every other day

c) May irrigate (enema)

d) Regular diet

e) May wear small patch, not pouch

b. Ileostomy, small intestine (ileum)

1) Continuous liquid stool with large acidic content

2) About 1000-1500 ml output every day

3) Empty pouch every 2-4 hours or when one-third full

4) Watch skin for irritation if there is leakage

5) Very special diet

6) Fluid and electrolyte problems

c. Jejunostomy, small intestine (jejunum)

1) Similar to ileostomy

2) Liquid output that drains 2000-3000 ml/ day

3) May attach to a Foley catheter bag to help drainage

4) Need IV nutrition to meet nutritional needs, as very little absorption takes place

Explain the purpose of an ostomy and the care of patients/residents with an ostomy.

D. Ostomy care

1. Equipment

- a. Soap and water
- b. Bag or pouch
- c. Wafer
- d. Wash cloth or paper towels
- e. Gloves
- f. Many different types of appliances
 - 1) One piece is a wafer and pouch together and cannot be reused
 - 2) Two piece
 - a) Wafer that lasts 5-7 days and cannot be reused
 - b) Pouch can be taken off and emptied and cleansed and reused multiple times

2. Emptying of pouch

- a. Check pouch every 2-4 hours and do not let the pouch get more than one-third full
- b. If the pouch is reusable; empty and rinse over toilet with water, dry and reapply
- c. Make sure “seal” is tight
- d. Observe contents of bag - color, amount, consistency, and odor
- e. Report complaints of discomfort

3. Skin care

- a. Wash skin well with soap and water, and dry well
- b. Shave hair
- c. Observe skin around stoma for redness and irritation

4. Odor management

- a. Everyone’s stool smells; ostomy patients/residents are no different
- b. There are sprays, tablets, etc. to reduce the odor

Explain the purpose of an ostomy and the care of patients/residents with an ostomy.

E. Nurse Assistant's role

1. Assist with personal hygiene
2. Provide for privacy
3. Change appliances
4. Emptying ostomy bag
5. Provide skin care
6. Use standard precautions
7. Encourage patient/resident to assist in the care
8. Reinforce the teaching plan and material shared by the licensed nurse
9. Be aware of cultural differences in attitude about ostomies and their care
 - a. Privacy
 - b. Shame
 - c. Body image